

Africa Health Workforce Agenda: Plan, Train, Retain, 2026 - 2035

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ACKNOWLEDGEMENTS

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FOREWORD

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ABBREVIATIONS AND ACRONYMS

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EXECUTIVE SUMMARY

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CHAPTER 1. SETTING THE SCENE

The African Union (AU) Heads of State have set a bold vision for the continent's future with Agenda 2063: The Africa We Want¹. Aspiration 1 of this agenda envisions a prosperous Africa driven by inclusive growth and sustainable development². A key pillar of this vision is the development of Africa's human and social capital through a revolution in education and skills, as well as the expansion of access to quality healthcare services. By 2063, the AU foresees an Africa where "the African population will be healthy, well-nourished, and enjoying a life expectancy of over 75 years", supported by integrated and comprehensive health services and infrastructure. These services will be available, accessible, affordable, acceptable, and of high quality.

Human capital development is undeniably a *sine qua non* for inclusive growth and economic advancement. However, the global COVID-19 pandemic significantly disrupted progress, reversing human development by half a decade to 2015 levels. In 2022, Africa's Human Development Index (HDI) was 18.5% below the global average (0.547 vs. 0.732), highlighting the urgent need for renewed investment in human capital.

A health workforce that has sufficient, well-trained, responsive, motivated, productive and equitably distributed health workers is the cornerstone of any health system and is essential to ensuring the provision of good quality health services, improving population health, ensuring UHC and the achievement of the Sustainable Development Goals (SDGs) (World Health Organization, 2016). The Global Strategy on Human Resources for Health (GSHRH): Workforce 2030 aims to accelerate progress towards achieving UHC and the SDGs by ensuring equitable access to health workers within strengthened health systems (World Health Organization, 2016). It emphasizes the need to optimize the existing HWF and to continue making adequate investments in education, recruitment, development, motivation, and retention, as well as ensuring adequate availability and a fair distribution of health workers, with an appropriate skill mix. Figure 1 below presents the GSHRH objectives.

¹ <https://au.int/en/agenda2063/overview>

² <https://au.int/en/agenda2063/aspirations>

Figure 1: Global HWF commitments for 2030: 4 key objectives



In 2017, the African regional framework for the implementation of the GSHRH was developed (WHO, 2017). Ministers of health adopted the framework as a regional blueprint providing contextual guidance on specific priority interventions, indicators, and milestones for 2020 and 2030.

Figure 2: African Region HWF commitments for 2030

Targets	
By 2022	By 2030
(a) All Member States have institutional mechanisms in place to coordinate HWF agenda.	(a) All Member States have reduced at least by half inequalities in access to a health worker.
(b) 50% of Member States have regulatory mechanisms in place to promote HWF safety and adequate oversight of the private sector.	(b) All Member States have reached at least 80% of the course completion rates in health training institutions.
(c) At least 50% of Member States have established accreditation mechanisms for health training institutions.	(c) All Member States have developed an HRH investment plan.
(d) At least 50% of Member States have an HRH investment plan.	(d) All Member States have established accreditation mechanisms for health training institutions.
(e) At least 50% of Member States have developed National Health Workforce Accounts (NHWA), and HWF observatories and registries.	(e) All Member States have regulatory mechanisms to promote HWF safety and adequate oversight of the private sector.
	(f) All Member States have established NHWA, and HWF registries and observatories.

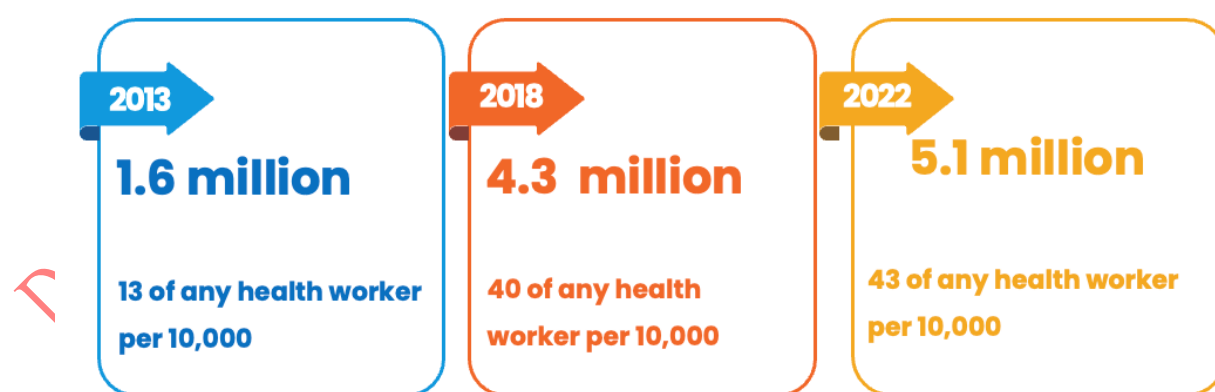
CHAPTER 2: THE STATE OF HEALTH WORKFORCE IN AFRICA

During the 62nd Regional Committee meeting held in Luanda, Angola, in November 2012, the Ministers of Health adopted a Regional Roadmap for Scaling up HRH for Improved Health Service Delivery in the African Region (2013-2025). Further to the commitments in the Roadmap, the 67th Regional Committee in 2017 adopted the African Regional Framework for Implementing the Global Strategy on Human Resources for Health: Workforce 2030. These commitments have resulted in intensified support to countries to develop plans and investments leading to notable progress in health workforce development over the last decade alongside improvements in service coverage.

The African Region recorded a notable increase in its health workforce stock and Density within the last decade.

The Africa region has recorded an increase in the stock of health workers from 1.6 million in 2013 to 4.3 million in 2018 and then 5.1 million in 2022, including some 850,000 community health workers. As of 2022, the African Region had 27 doctors, nurses, midwives, dentists, and pharmacists per 10,000 people on average, a more than two-fold increase from 11 per 10,000 people in 2013 and a 14% improvement from 2018. This highlights concerted efforts and investments in the improving availability of qualified and skilled health workers to provide much-needed health services to the population in need, giving us hope for the future.

Figure 3: A 3-fold increase in the stock of all health workers in 10 years

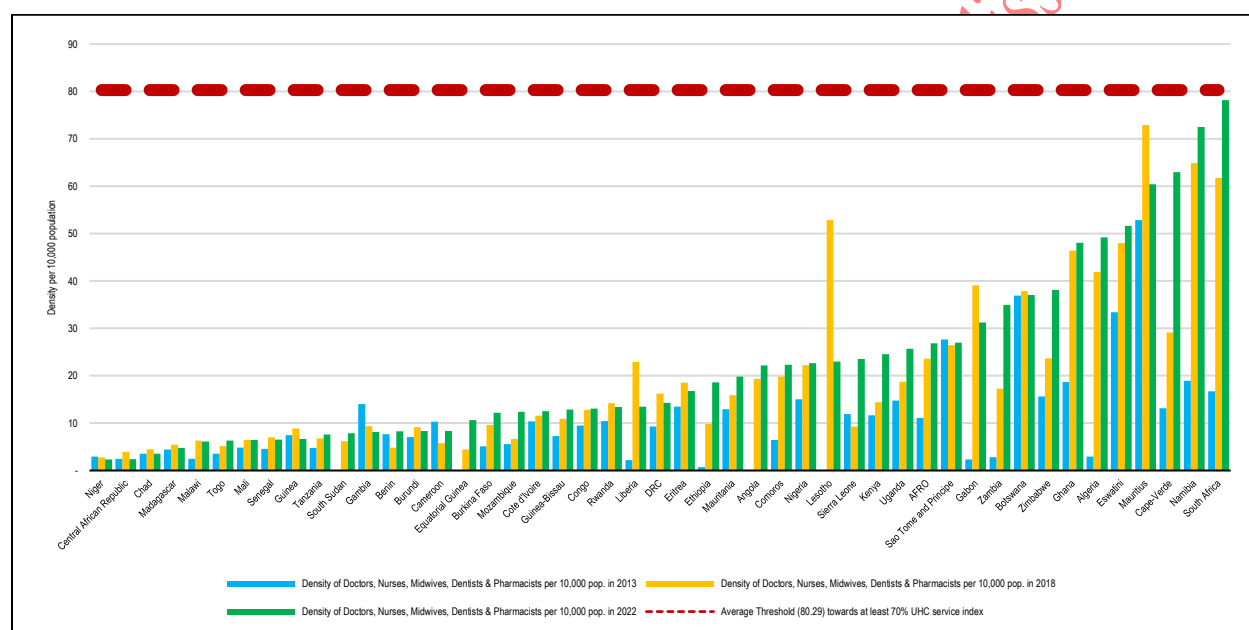


However, the progress made was uneven with countries of different context in different pace or even trajectories. For example, 12 countries³ that are relatively better off have 12 times more

³ Gambia, South Sudan, Tanzania, Senegal, Mali, Togo, Malawi, Madagascar, Guinea, Chad, Central African Republic and Niger

health worker densities than 12 countries⁴ with the lowest. Thirty-seven out of 47 countries (79%) increased their number between 2018 and 2022, but 10 countries recorded no improvement. Twenty-nine countries (62%) improved their densities per 10,000 population between 2018 and 2022, while 18 (38%) countries lost momentum. Eight (17%) countries⁵ increased their stock, but this improvement was outpaced by their population growth.

Figure 4: Density of doctors, nurses, and midwives per 1000 population in 2005 and 2018 in the African Region showing SDGs and MDGs index density threshold per 1000 population (pop)



Source: WHO/AFRO, 2024 – A decade Review the Health Workforce in WHO African Region. **The graph is not showing Seychelles (an outlier) which had a density of 54.8 in 2013, 195.92 in 2018 and 242.01 in 2022*

Africa's health workforce is predominantly young and female

Overall, the African health workforce is female, with women accounting for 72% of the health workers in the Region. Nonetheless, the women are concentrated within low paying occupations such as community health workers, midwives and nurses. There has been, however, a notable improvement as 35% of the medical doctors in 2022 were female compared to 28% in 2019. Also, Africa has the highest proportion of male nurses of 34% compared to a global average of 15%.

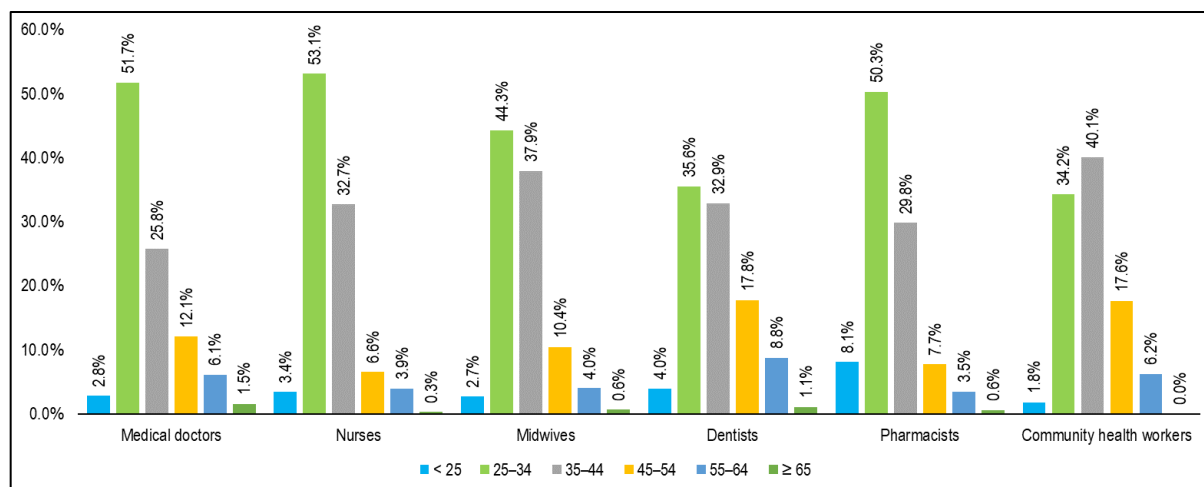
Health workers in the region are relatively young, with 82% under 45 years. Nurses lead with 89%, followed by pharmacists (88%), midwives (85%), medical doctors (80%), community health

⁴ Seychelles, South Africa, Namibia, Cabo Verde, Mauritius, Eswatini, Algeria, Ghana, Zimbabwe, Botswana, Zambia and Gabon

⁵ Botswana, Burundi, DRC, Malawi, Mali, Niger, Rwanda, and Senegal

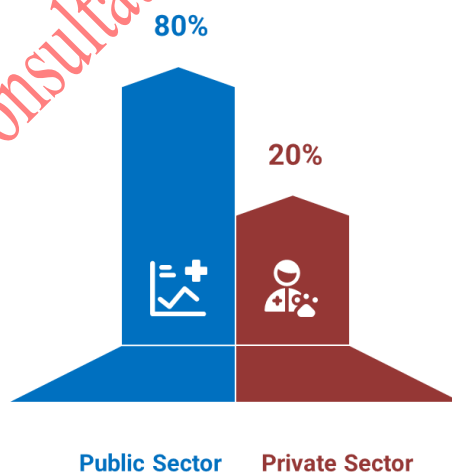
workers (76%), and dentists (72%). Less than 10% are over 55, except for nurses, dentists, and community workers in high/upper-middle-income countries, and medical doctors in lower-middle-income economies.

Figure 5: Age distribution of selected occupations in the region.



Looking at the data, it is showed that majority of health workers are working in the public sector, while the private sector employs 20%. The private sector employment varies from 51% to 3%, respectively in Namibia where the private sector plays a significant role, and the Gambia where the public sector is the main employer (97%).

Figure 6: Public vs Private sector distribution



Africa's health workforce is increasing but quality and competencies remain a challenge

Africa is gathering momentum in its health workforce development, recording an expansion by 70% in its capacity to train health professionals, increasing from 150,000 graduates in 2018 to over 255,000 in 2022 from more than 4,000 health training institutions in the region. About 10% of these institutions offer medical training, while over 50% train nurses and midwives. Thus, there are at least 400 medical schools reported in the African region (Rosenberg et al., 2024) producing 28,600 doctors annually compared to 168 schools that produced 6000 doctors per annum in 2005 (WHO/AFRO, 2006, 2021b, 2024a). Additionally, at least xxxxxx nurses/midwives are being produced annually compared to 26,000 in 2005 (Asamani et al., 2024b; Okoroafor et al., 2022a). Of the available education capacity, the private for-profit sector contributes 34% and the private, not-for-profit 10%. The Region's stock of Community Health Workers increased by about 76% in 4 years, from 486,186 in 2018 to 853,500 in 2021 (African Union, 2021; WHO/AFRO, 2021c).

Despite these improvements, there is uneven distribution and inadequate training and education capacity and quality. For example, almost 66% of the regional training capacity is concentrated in Nigeria, DRC, Ethiopia, Tanzania, Ghana and South Africa (WHO/AFRO, 2024a). Even in these countries, infrastructure and tutors are still inadequate. The COVID-19 pandemic exposed serious gaps in the Training in emergency and critical care and other public health workers such as field epidemiologists. For example, only 9 out of 20 countries could train emergency and critical care doctors in East and Southern Africa, while pharmacy and laboratory science specializations were almost not readily available (Asamani et al., 2022a).

Health workers in the African region face significant challenges related to competencies for quality health service delivery. Across a quarter of countries in the region, some assessments have revealed that on average, diagnostic and treatment accuracy among health workers is 56%, with nearly 70% scoring 50% or less in these crucial areas (Daniels et al., 2024; Sheffel et al., 2024). Competency levels are lower among nurses and other health workers compared to doctors, and public-sector professionals tend to perform better than those in the private sector. In the context of maternal and child health, health professionals are able to correctly diagnose 64% (95% CI 62% to 65%) of the clinical problems and in 45% (95% CI 43% to 46%), their treatment plan was aligned with the correct diagnosis. Despite a high rate of almost 30% absenteeism, poor clinical knowledge is a greater constraint in care readiness than drug availability or health workers' absenteeism put together (Di Giorgio et al., 2020). These competency gaps are contributing significantly to 48% of healthcare amenable deaths that are due to poor quality of services in Africa (Kruk et al., 2018).

Some of the competency gaps of health workers are traced to training curricula that are not competence-based, compromising the quality of health workers being produced. In many countries, it is rarely formally acknowledged but the curricula health professions education (HPE) institutions are outdated, with content and approaches for implementation not aligned with the current health needs of society (McGivern et al., 2021; McGivern et al., 2021; Moller et al., 2022;

WHO/AFRO, 2021d). In some countries, this stems from many HPE institutions still using curricular models from colonial times, reflecting an outdated approach to education (Mnguni, 2024).

Curriculum changes are often driven by societal reforms, professionalization needs, epidemiological transitions, and external pressures (Mnguni, 2024). Major curriculum transformations are often financed through external partnerships and foreign aid, yet these efforts face sustainability challenges due to limited domestic investment. Additionally, issues such as inadequate infrastructure, lack of leadership, and insufficient capacity among educators contribute to a phenomenon known as curriculum drift. This drift, observed in several HPE institutions across Africa, reflects a weakening of mechanisms designed to sustain curricular advancements. As a result, initial investments—whether educational, financial, or technological—are undermined, ultimately hindering the long-term success and impact of curriculum innovation (Nyoni and Botma, 2020).

Nevertheless, some advancements have been made, particularly in nursing, where countries like Angola, Ghana, Kenya, Lesotho, Malawi, Rwanda, South Africa, and Zambia are adopting competency-based education (Nyoni and Botma, 2020; WHO/AFRO, 2022) following a regional prototype curricula developed by WHO (World Health Organization. Regional Office for Africa, 2016a, 2016b, 2016c). Even though 80% of countries report be using the African Regional Prototype Competency-Based Curricula (WHO/AFRO, 2024a), this self-reported usage is not based on independent benchmarking to ensure alignment with competency-based models at country and institution level. In some instances, the changes to curricula are minimal compared to the broader need for transformation (Priestley, 2010; Ramrathan, 2016), thus, necessitating strategies to support the appropriate localisation and adoption of regional curricula prototypes to ensure fidelity in implementation.

Educators within HPE institutions drive the development competent health workers, but their limited numbers and capacity remains a longstanding challenge. In most countries, there is a shortage of educators in HPE institutions, including in clinical environment where students are place for experiential learning, most of the educators do not have the necessary educational qualification, experience and expertise. It is argued that most health professions educators are hired for their disciplinary expertise regardless of their lack of educational expertise and often not given opportunities to build the necessary educational expertise in a formal training (Frantz et al., 2015). When educators are not qualified in education, they struggle to understand, adopt and implement educational practices such as competency-based education. WHO has developed resource to address this challenge for nurse and midwives educators (WHO, 2016b, 2013), but a scale-up to all educators of the health profession is urgently required.

Interventions to improve the educational capabilities of educators in the African region remain fragmented, opportunistic and insufficient. A handful of HPE institutions have developed HPE units- who are mandated to support educators through localised faculty development initiatives, and faculty development initiatives being predominately once-off

workshops, webinars or short courses with poor mechanism for monitoring impact in practice. There is also a tendency for faculty development initiatives to have less priority or emphasis on clinical educators (Makoni et al., 2024; Van Schalkwyk et al., 2020). In addition, national and regional initiatives, often initiated and supported by partner organisations which often collapse after donor funding ends due to lack of sustainability and local investment (Keiller et al., 2022).

Quality Assurance and Regulation of Health Professions Education is reported to have improved, but strengthening their capacity for impact is crucial. More than 80% of countries in the African region report having accreditation frameworks for health professions education (Okoroafor et al., 2022a). However, the practical implementation and impact of these mechanism on the quality of education remain undocumented in some context (Nakidde et al., 2025) while in others health practitioners perceive their relevance as weak (Mcgivern et al., 2021). Over the years, WHO has made tremendous efforts to standardize regulatory and accreditation mechanisms for health workers (WHO, 2009; WHO/AFRO, 2016d). There are also continental, regional and sub-regional efforts by various stakeholders to harmonise regulatory mechanism and curricular towards reciprocal recognition of qualifications (SADC, 2021; WHO/AFRO, 2024).

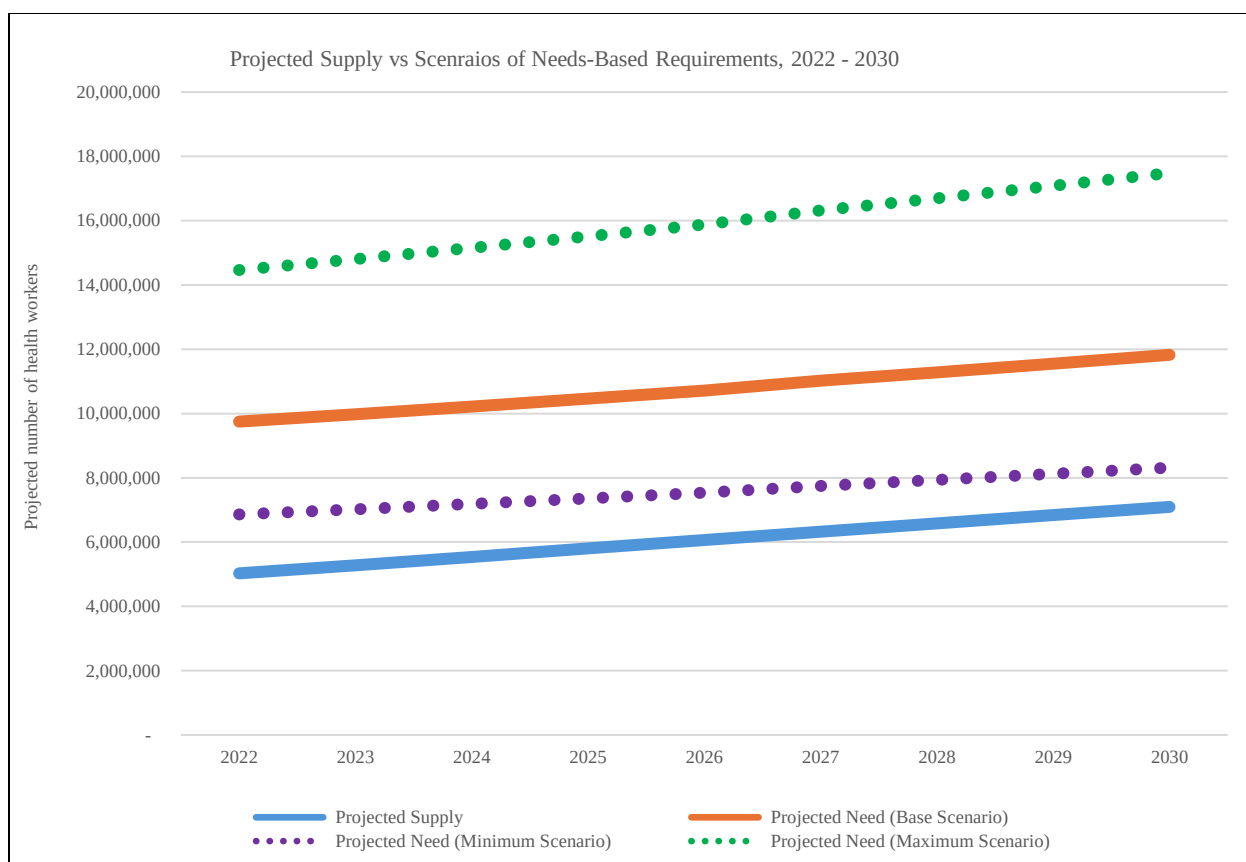
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Africa's faces a paradox of needs-based shortage and raising unemployment of health workers

Despite the progress made, the central problem facing the Region is the **shortage of competent and motivated Health Workers with the appropriate skills mix**. With the Region having less than 5% of the global workforce stock, its health workers are dealing with almost 24% of the global disease burden and at least 100 episodes of graded emergencies per year. In 2024, WHO AFRO undertook an extensive workforce modelling using a novel epidemiological needs-based methodology that links disease burden and population demographics to the essential health service package and workforce productivity required to address them (WHO/AFRO, 2024c).

Despite the strides in increasing the stock of health workers, needs-based projections based on the epidemiological profiles and demographic profile of each country indicate a potential aggregate shortfall of 6.1 million health workers by 2030, which is necessary to effectively deliver comprehensive health promotion, disease prevention, treatment, rehabilitation, and palliation (WHO/AFRO, 2024c) and accelerate the pace of UHC by some 53% (Asamani et al., 2024a). In the global context, the African region carries more than 50% of the global workforce crises, and 40 out of 55 countries identified globally as having vulnerabilities and needing Safeguards and Support are in the African continent (Boniol et al., 2022). **Figure xxx** illustrates the projected aggregate health workforce supply and scenarios of needs-based requirements. Projection shows no feasible scenario(s) to completely address the quantitative shortage of health workers by 2030, thus requiring a strategic approach to progressively accelerate the pace of progress in planning, education, employment and retention.

Figure 7: Projected Supply vs Scenarios of Needs-Based Requirements, 2022 - 2030



Labour market failures have exacerbated health workforce unemployment, unsatisfactory working conditions and failure to retain.

Investments in HWF recruitment have not kept pace with the outputs from training (supply), leading to a growing paradox of surplus health workers who fail to find jobs despite a need-based shortage at the frontlines of service delivery. Health labour market analysis in 12 countries suggest that nearly 27% of the trained health workers remain unemployed, underscoring a mismatch between training outputs and actual job creation. For instance, 27% of nurses in Lesotho (MOH, 2021), 22% of all health workers in Ethiopia (FMOH, 2020), 14% in Kenya (Okoroafor et al., 2022b), 30% in Malawi (Berman et al., 2022), 19% in South Africa (Hlayisi, 2022), and more than 40% in Ghana and Zambia (MOH, 2024a, 2024b), are unemployed, but vacancy rates in these countries remains high. Also, an analysis by the World Bank (World Bank, 2020) shows that trained but unemployed nurses in the East, Central and Southern Africa sub-region could reach 236,281 (or 28.5%) by 2030. WHO AFRO's modeled estimates suggest that the unemployment requires a 43% increase in current funding levels dedicated to health workforce employment (Asamani et al., 2022b, 2025).

Sluggish private sector absorption of health workers and reduced external aid has increased vulnerabilities and the threat of health workforce outcomes. The private sector's absorption is estimated 22% of the HWF, but this varies markedly across countries (Asamani et al., 2022b, 2022c; Okoroafor et al., 2022b; WHO/AFRO, 2021c), despite a higher proportion of the private sector's contribution to the training throughput (Okoroafor et al., 2022a), a trajectory that leaves a cumulative absorption capacity in the region at 70% (WHO/AFRO, 2024a, 2024c) - and leaving approximately 30% of newly trained health workers could become unemployed or underemployed. Similarly, official development assistance (ODA) has been contributing to 25% of current health spending in most countries of the region (WHO/AFRO, 2024d). However, in 2025, waves of announcement of both abrupt and progressive external aid transition has heightened the financing vulnerability of health systems and potentially weakening their health workforce absorption capacity (Zurn et al., 2025), which was already sub-optimal at 70%. The OECD expects external aid cuts of 9-17% (OECD, 2025), and if it proportionately impacts on health and the health workforce spending, the cuts could significantly reduce countries' ability to hire new graduates, exacerbating Africa's projected needs-based shortage to 6.6 million if absorption capacity reduces by 20%, and to 6.4 million if absorption capacity reduces by even 10%.

The Africa region is failing to retain its experienced and sometime specialised health workforce due to several push and pull factors related to the labour market forces and opportunities for professional growth. The Africa face several challenges related to suboptimal working conditions, with at least 14 countries experiencing an average of four (4) instances of health worker industrial unrest or strikes annually since 2018. At the same time, brain drain continues to be a significant issue, with one out of every ten doctors or nurses trained in Africa now working abroad (WHO/AFRO, 2024a) and 42% intending to emigrate sometime in the future. Healthcare worker migration is a critical challenge for African countries. In Nigeria, over 3,000 doctors and 11,838 nurses and midwives migrated in 2022, with similar trends in Kenya, Uganda, Ghana, and Zimbabwe. Ghana saw a 232% increase in nurses seeking migration verification from 2020 to 2022. Zimbabwe, a major source of UK health workers, had its migration rate nearly double to 6.5% in 2021, though government interventions reduced it to 3.4% by 2022. Addressing this crisis requires strategic investment and policies to retain and support the healthcare workforce across the continent.

Sub-optimal health worker productivity and performance is amplifying the impact of the shortages of health workers across many African countries

Despite a 13% improvement in efficiency between 2014 and 2019, African health systems remain only 77% efficient. This means that about US\$1 in every US\$5 spent on health is lost to technical inefficiencies, including health workforce management. This plays a significant role in the existence of ghost workers on payrolls, absenteeism and sub-optimal performance, for example.

For example, it is reported that the average level of health worker absenteeism is high – about **35%** (range: 14 - 54) (Sheffel et al., 2024), which is undermining health worker productivity and performance, leading to 20% overall technical inefficiency of the African health system (Nabyonga-Orem et al., 2023; WHO/AFRO, 2021e) and ultimately adversely impacting health outcomes. This has been partly attributed to a complex mix of poor remuneration and incentives as well as weak performance management systems, poor supervision, lack of performance standards and lack of productivity improvement tools and metrics (Frimpong et al., 2011; WHO, 2016). Skill mix imbalances which have also been implicated in the myriads of factors affecting productivity inhibit efficient utilisation of the health workforce (Buchan and Dal Poz, 2002; Sander et al., 2015).

While productivity is a critical measure of health worker performance, governments have not institutionalized its measurement in the health sector to inform performance management, pay policies and resource allocation decisions. Also, most low—and middle-income countries have not embraced fully performance-based reward mechanisms that would induce productivity improvements amongst individual health workers and engender efficiencies in health facilities. One of the main challenges has been the absence of standardised metrics and validated tools to assess health worker productivity and performance.

Countries in the African region are at different stages of developing their health workforce governance and leadership.

The existence of formal institutional mechanisms for health workforce planning, development, management, policy coordination is indispensable for addressing health workforce challenges. Dedicated HRH units within health ministries play a vital role in strategic workforce management, including forecasting staffing needs, guiding investments in education and recruitment, and standardizing training programs to ensure healthcare professionals meet quality care standards. These structures enhance coordination and sustainability in workforce development, ultimately improving health system performance and population health outcomes (Effa et al., 2021; Lim and Lin, 2021; Okech et al., 2021).

In the African region, thirty-three (33) countries have established HRH units within their Ministries of Health to lead the health workforce agenda, with the HRH heads often being a part of the senior management, ensuring high-level representation of health workforce issues (WHO/AFRO, 2024a). However, 76% (25 countries) have decentralized these units to the subnational levels. The functions of these units vary within countries and include HRH planning, personnel administration, HRH information system, HRH policy development, and training and development (Afriyie et al., 2019). Furthermore, 29 countries have put in place institutional mechanisms to coordinate an intersectoral health workforce agenda, 24 countries had mechanisms and models for health workforce planning, and 22 countries had national education plans for the health workforce that were aligned with national health plans and strategies. Nevertheless, nearly

40% of countries lacked institutional models for assessing and monitoring staffing needs for health service delivery (Afriyie et al., 2019).

Box xxx: Transforming Health Workforce Governance in Africa: Exploring Leadership Models

Across the African region, it is observed that countries mostly three different mechanisms for workforce leadership and governance, particularly in the health sector, namely, (1) the Civil Service Model, (2) Dual Civil Service – Technical Model, and (3) Integrated (Hybrid Model).

Pure civil service model: In some countries, the human resources for health department or division in the Ministry of Health is completely under the control of the civil service. All directors and other staff are posted by the civil service and transferred from one ministry to another periodically. In these countries, longevity within the human resources for health directorate is very short, characterized by a high attrition rate. Directors and staff of the human resources unit are frequently changed, sometimes coinciding with the short tenure of other principal officers, such as the minister or permanent secretary. As a result, continuity of strategic human resources for health functions tends to suffer, with institutional memory often lost about processes that started earlier. Additionally, the specific capacity required for functions within the health sector, which are quite peculiar compared to other sectors of the economy, is often lacking.

Dual Model (Civil Service – Technical): To overcome the limitations observed in the pure civil service model, some countries have adopted a dual model. In this model, there are two units of human resources for health. The management function is performed by the HR unit nested in the administration department of the ministry, while the strategic functions of planning, monitoring and evaluation are nested within another department that covers health systems functions or the health planning and policy department. In this dual model, the longevity of strategic functions in the human resources for health area is largely maintained, as those assigned to these functions have specific occupations that are not usually transferred to other ministries as frequently as civil service staff. However, it has been observed in some countries that the two units rarely plan and execute the human resources for health agenda jointly. In practice, the HRH unit performs health system-related functions, including planning, monitoring, evaluation, and integration of HR into health sector policies and plans. Meanwhile, personnel management is handled by the HR department, often under another director. This lack of synergy leads to strategic policies and plans not being translated into operational policies and guidelines, affecting equitable distribution, appropriate compensation, in-service training, and education.

The Integrated (hybrid) model: This is where the human resources for health function is established as a full directorate headed by one director, with departments under it to perform the strategic and management functions. Some departments perform strategic workforce policy and planning functions, others handle training and education, personnel management, and labor relations functions. In this hybrid model, there is more synergy between these functions. Strategic planning, monitoring, and evaluation functions are often protected from routine civil service rotations across ministries, ensuring continuity of structured functions. Personnel management functions, however, tend to be amenable to transfers across the civil service.

Although these three distinct models have been practiced in various countries, there has not yet been a formal evaluation of the extent to which each of these models impacts overall service delivery and health outcomes. Nevertheless, countries that practice the hybrid model tend to have more responsive policies and better outcomes compared to those with the dual model and the pure civil service model. This is due to the short tenure of HRH leadership in the pure civil service model, which hampers the establishment of relationships and continuity required for effective HRH management. Further research and evaluation are needed to establish the relationship between these models and health outcomes, to explore harmonization and strengthen HRH structures in African countries.

Increasing availability of evidence-based national health workforce strategies in the African region is setting a positive tone for the future of the health workforce in the African Region.

Evidence-based human resources for health policies and strategic plans establish clear national priorities for health workforce with specified objectives, roles, and responsibilities for workforce development, ensuring that all activities, including planning, training, recruitment, deployment, retention and regulation, are aligned with national health priorities and informed by the populations' health needs (Nyoni et al., 2022). In 2022, twenty-four (24) countries⁶ out of 30 that reported in the National Health Workforce Accounts Platform indicated that they have HWF policies to guide the overall direction and management of health workers. In addition, twenty-two (22) countries⁷ out of 35 had reported having HWF strategic plans or investment plans. A majority (63%) of the countries reported having up-to-date HWF strategic plans or investment plans as of 2022, a slight decrease from 70% in 2018. Additionally, five countries were in the process of developing their HWF strategic plans. A further five countries (Burkina Faso, Cameroon, Congo, Guinea-Bissau and Liberia) indicated that they were in the process of developing their HWF strategic plans.

Only half of African countries have reported having funds to implement their health workforce plans – hence despite most countries having strategic plan, in the implementation of these plans, the data shows a mixed picture.

Consistently, most countries have HWF policies and strategic plans in place, only about 50% of these countries have committed to increasing their budgetary allocation to support the implementation of these plans (WHO/AFRO, 2024a, 2021b).

Regarding the costing and linkage of HRH plans to development plans, the data shows that among countries with strategic plans, 25 had their plans costed for implementation, and 28 linked their plans to the national development plan. Furthermore, the plans of 30 countries were informed by the national health policy and strategic plans, and 26 countries linked their strategic plans for HRH to the global strategy for HRH and the Africa regional implementation framework. Twenty-five countries included HRH needs projections in their HRH strategic plans, with only eight countries covering both the public and private sectors.

Data from 29 of the 47 Member States shows that more than 75% of countries (n = 22) have laws and/or regulations on the working conditions of health workers.

⁶ Angola, Benin, Burundi, Chad, Comoros, Cote d'Ivoire, DRC, Eritrea, Eswatini, Ethiopia, Gambia (The), Ghana, Guinea, Kenya, Mali, Mauritania, Mozambique, Namibia, Nigeria, Senegal, Sierra Leone, Tanzania, Zambia and Zimbabwe

⁷ Angola, Benin, Botswana, Chad, Comoros, Cote d'Ivoire, DRC, Eritrea, Eswatini, Gambia(The), Ghana, , Kenya, Mali, Mauritania, Mozambique, Namibia, Nigeria, Senegal, Sierra Leone, South Africa, Tanzania, Zambia and Zimbabwe

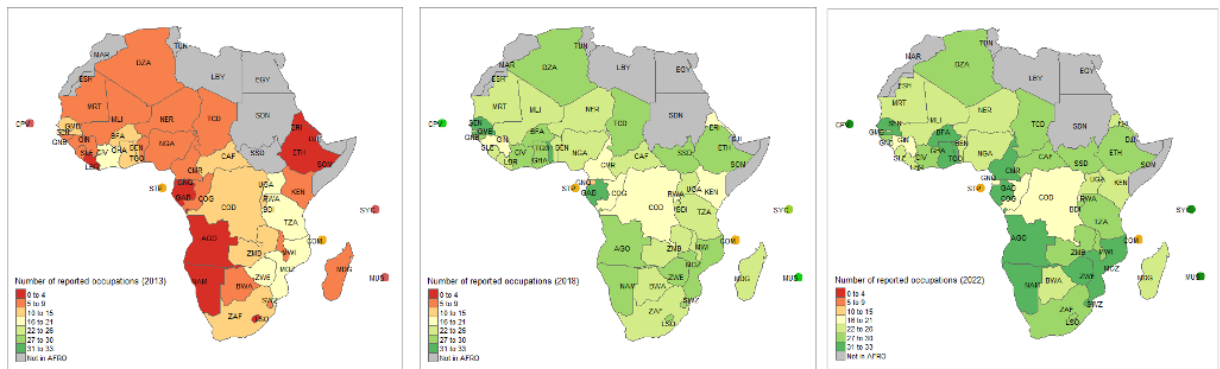
Regulatory mechanisms for health worker education and practice are critical to ensure quality health care delivery and patient safety (Burdick and Dhillon, 2020; Okoroafor et al., 2022a). Accreditation bodies set national training standards and play an important role in evaluating and approving training institutions against established standards, guaranteeing that curricula produce graduates with the competencies required to meet evolving health needs. This quality assurance prevents variability in graduate skills and promotes continuous improvement in teaching and learning environments. Similarly, licensing and registration systems within professional regulatory bodies verify individual practitioners' qualifications and conduct, deterring unqualified practice and providing avenues for disciplinary action when standards are breached (Mahat et al., 2023). Regulatory bodies also enforce codes of conduct and scope-of-practice regulations and uphold ethical norms. Furthermore, regulation standardizes scopes of practice and fosters managed workforce mobility.

Regulatory mechanisms for health workforce safety and oversight of public and private sectors is one of the crucial steps towards the health workforce development with WHO setting a target for 50% of all 47 Member States to have set up accreditation bodies by 2022 and all by 2030. As of 2022, 39 countries (83%) reported that they had functional regulatory bodies of various nomenclatures that monitor and enforce the standards that regulate the education, practice, safety and conduct of healthcare professionals in the public and private sector.

With years of efforts to improve health workforce data systems, many countries are able to track the stock of their health workforce and use data for decision-making.

In 2013, countries could track and report data for 26% of occupations across all 47 countries. This improved significantly to 75% in 2019 and 81% in 2022, thanks to all countries in the Region adopting to implement the national health workforce account. **Since 2017, 25 countries have been supported by WHO in conducting health labour market analyses**, which have informed policy reforms, stimulated job creation for health workers, and unlocked investments from government and partners.

Figure 8: Health Workforce Data Availability and Use, 2013 - 2022



Inadequate health workforce financing is amplifying the labour market failures

It is estimated that the level of health spending in Africa is lower than international benchmarks, leaving **US\$xxxx billion** additional investments to halve the 6.1 million shortage of health workers. In 2022, WHO Regional Office for Africa estimated financial requirement to employ the existing stock of health workers was US\$36.3 billion, whereas the available funding from all sources amounted to only US\$20.85 billion, leaving a financing gap of approximately 43%, assuming full employment of all trained health workers across the public, private, and overseas development assistance sectors (Asamani et al., 2025; WHO/AFRO, 2024c). Given the current levels of health workforce funding, addressing half of the existing health worker shortage over the next eight years will necessitate an investment of US\$120.41 billion. Of this total, US\$22.58 billion (18.75%) is required for the training of additional health workers, while US\$97.83 billion (81.25%) is needed to employ both newly trained health workers and sustain salaries of the current workforce. Although the total investment needed for health workforce development appeared substantial, it constitutes less than 2% of the combined Gross Domestic Product (GDP) of countries within the African Region. Many countries that have conducted comprehensive health labour market analyses, such as Central African Republic, Ethiopia, Eswatini, Ghana, Kenya, Lesotho, Mozambique, Uganda, and Zambia, have produced national estimates to guide policy dialogues and investment planning.

Countries in the African region have diverse realities of health spending prioritization and fiscal space potentials even though on average they allocate about 7% of the general government health expenditure on health (range: 2.1% to 15.3%). From these, an average of 43% (range: 12% to 71%) is often allocated to health workforce remuneration and wages, which determines the levels of employment of health workers in the countries. Four groups/typologies of health and health workforce expenditure prioritization can be deduced (Asamani et al., 2025), as shown in **Figure 8**.

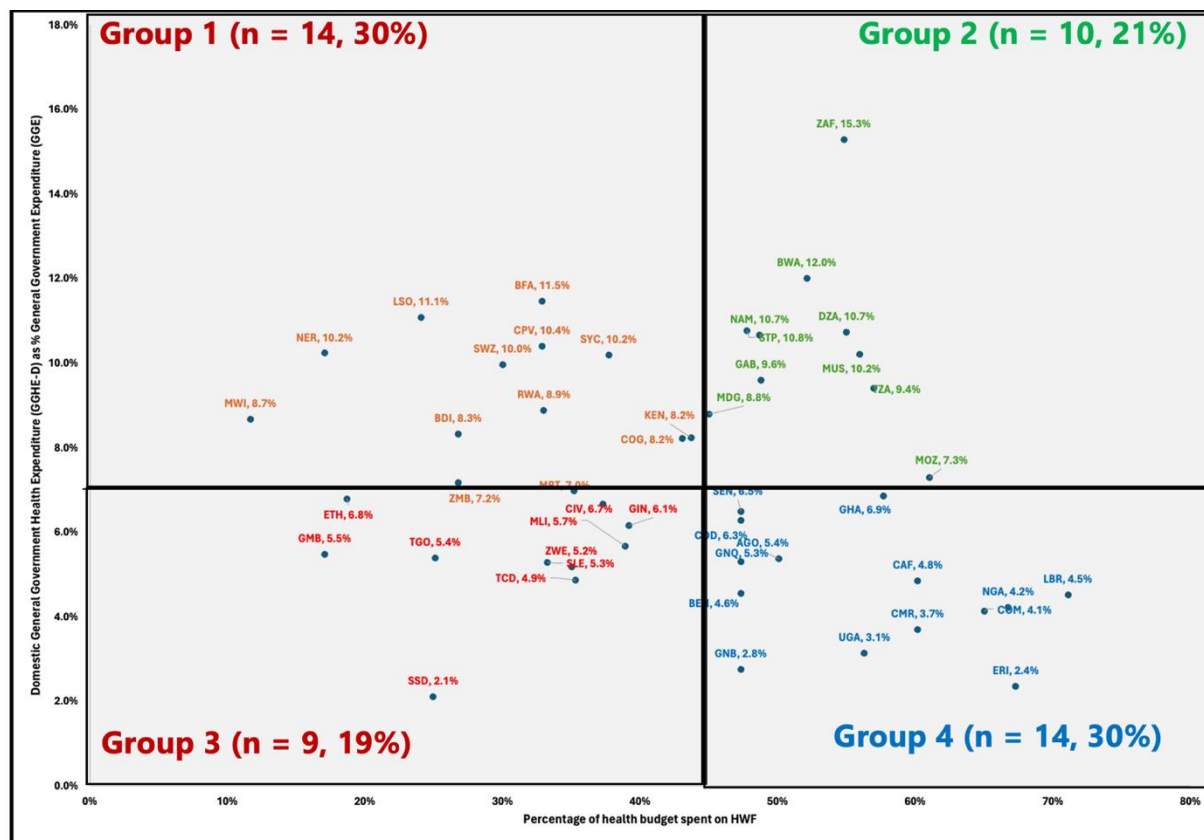
In the first group (countries with High Health Spending, Low Workforce Prioritization), countries allocate more than the regional median to health within government spending (>7%) but allocate less than the median share to the health workforce within health budgets (<43%). Representing 30% of countries (n=14), opportunities to expand health workforce investments mainly come from improving prioritization within existing health budgets. Advocacy efforts should also focus on increasing overall health budgets or mobilizing external aid where appropriate.

The second group (countries with High Health Spending, High Workforce Prioritization) includes countries that exceed the regional median in both government health spending (>7%) and prioritization of the health workforce within health budgets (>43%). Comprising 21% of countries (n=10), these “frontier” countries could enhance health workforce investments primarily through improving allocative efficiency of current spending, alongside advocacy to increase health budgets or secure external support as needed.

The third group (countries with Low Health Spending, Low Workforce Prioritization) consists of countries that allocate below the regional median to health in government spending (<7%) and also allocate less than the median share of health budgets to the health workforce (<43%). Accounting for 19% of countries (n=9), these nations face significant constraints and require strong political commitment to increase both overall health spending and the proportion dedicated to the health workforce.

Finally, the fourth group (countries with Low Health Spending, High Workforce Prioritization) includes countries that spend less than the regional median on health within government budgets (<7%) but allocate a higher than median share to the health workforce within health spending (>43%). Comprising 30% of countries (n=14), opportunities to increase health workforce investments lie mainly in raising overall health budget prioritization or securing external development assistance targeted at the health workforce.

Figure 9: Prioritisation for Health and Health Workforce in the WHO African Region



Source: Asamani et al (2025) <https://doi.org/10.1177/11786329251320429>

Why is HWF still a worthwhile investment ... investing in the HWF has multiple returns for both health and the economy.

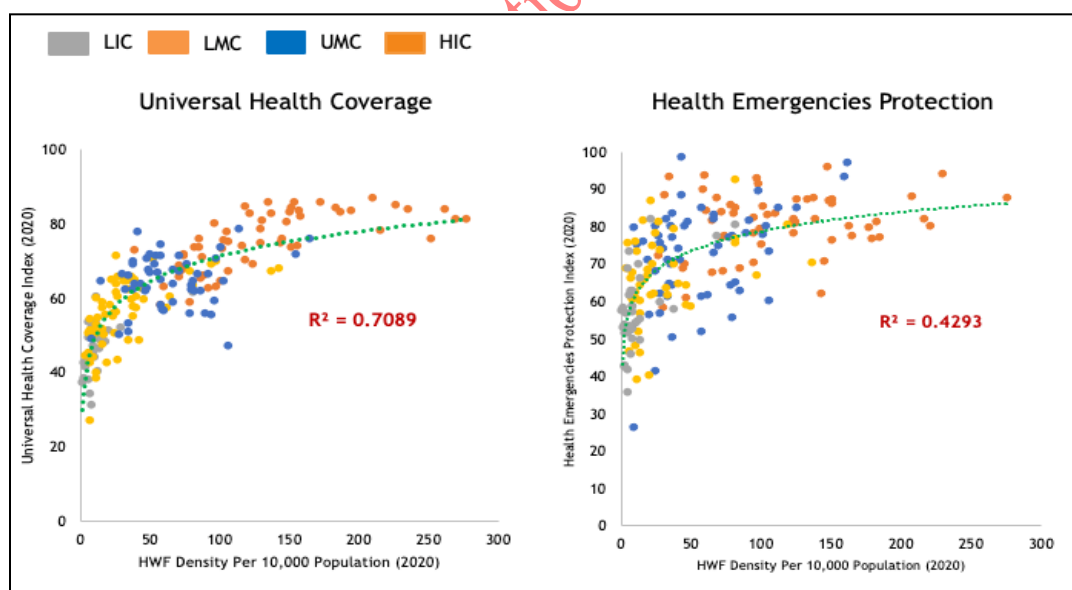
Investing in health has a direct, transformative impact on the three core dimensions of human development: a long and healthy life, access to knowledge (education and skills development), and a decent standard of living (economic growth). Specifically, investment in health workforce (HWF) development yields an impressive return of 1:9, with strong empirical evidence showing its correlation to improvements in key human development indicators. For example, HWF investment is linked to improvements in mean years of schooling (SDG 4.4, $R^2 = 0.58$), health service coverage and life expectancy (SDG 3.8, $R^2 = 0.55$), and overall human development ($R^2 = 0.59$).

Several studies have reported substantial health and economical gains as a result of investments in HWF (Ahmat et al., 2022; Anand and Bärnighausen, 2004; Liu and Eggleston, 2022; Robinson and Wharrad, 2001; World Health Organization, 2017). These include promotion in the achievement of SDGs and progress towards UHC, increasing life expectancy, reducing maternal mortality and child mortality and job creation, especially for youth and women and education.

There is clear nexus between HWF investments and the SDGs such as goals 4 on (quality education), goal 5 (on gender equality) and goal 8 (on decent work and economic growth). The report of the UN High Commission on Health Employment and Economic Growth (HEEG) demonstrated that investing in the HWF yielded a nine (9) fold return on investment (World Health Organization, 2017). Thus, for every dollar invested in creating decent employment for health workers, the potential return is about nine dollars. It also demonstrated that half of the global economic growth over the last decade resulted from improvements in health, noting that for every added year of life expectancy, the economic growth rate is boosted by 4%. For example analysis done in Namibia revealed that for every unit increase in essential health workers per 1,000 population, there is a corresponding improvement in overall life expectancy at birth by 2.78 which in turn accelerates growth rate of the national economy by 1.67% for every year of life expectancy added (MoHSS, 2019).

Health workforce density is highly correlated with UHC service coverage score and health emergency protection (HEP) (Figure 1); investments in the HWF are critical to driving improvements in UHC targets and health security. Diseases cost Africa some \$2.4 trillion annually in lost economic productivity across all sectors (World Health Organization, 2016). Thus, addressing the disease burden with effective health interventions is not only a fundamental human right but an economic investment which could propel GDP growth by an estimated 0.5% per annum.

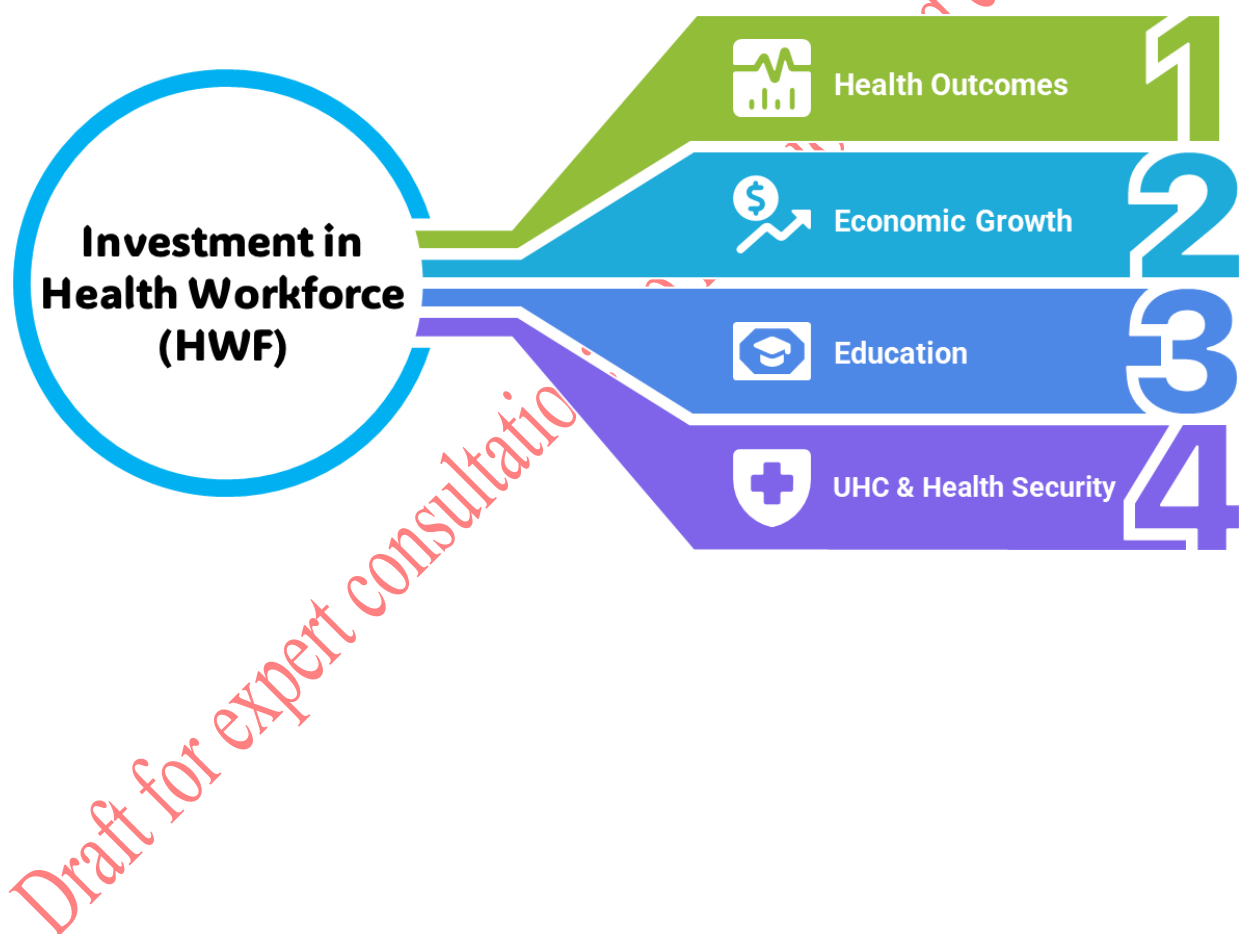
Figure 10: Correlation between HWF density, UHC and HEP index, 2020



Source: WHO, 2022

Eaneff et al., (2022) emphasised that **any investment strategy in global health security must account for financing for personnel**—identifying mechanisms to financially support the training, development, and ongoing salary requirements for those workers whose efforts underlie critical public health systems. The authors estimate that investment in HWF is beneficial across all domains of health security and that 66% of the amount of money needed for global health security need to be spent on HWF. This is also consistent with ‘price tag’ study for SDGs in which Stenberg et al., (2017) demonstrated that increased investment in HWF is critical to the achievement of SDGs and recommended that of the investment needed to achieving SDGs 3, more than 50% need to be spent on HWF employment (wages and salaries), and could reach 80% if the investment needed in training is taken into account.

Figure 11: HWF Investment outcomes



CHAPTER 3: KEY ACTIONS TO ACCELERATING SUSTAINABLE HWF DEVELOPMENT IN AFRICA

5.1 VISION

All people across the life course have access to quality essential health services they need, where and when they need them, without suffering financial hardship.

5.2 GOAL

The goal is to accelerate the equitable access to skilled and motivated health workers within strengthened health systems through additional employment of at least 1.5 million additional health workers across 47 Member States in the African Region. These health workers will contribute to the provision and utilization of high-quality people-centered health services that will culminate in attaining a UHC service score of at least 80 out of 100 by 2030, reduce maternal mortality to less than 70 per 100,000 live births, and under-five mortalities to less than 25 per 1,000 live births and improve the well-being of all including during emergencies and humanitarian situations.

Maternal mortality, under-five mortality and UHC index are strong measures of health system capacity and performance and collectively targeting them have a positive and catalytic externality in improving as part of the health system drivers.

5.3 THEORY OF CHANGE: DOING WHAT MATTERS TO ACHIEVE THE ULTIMATE

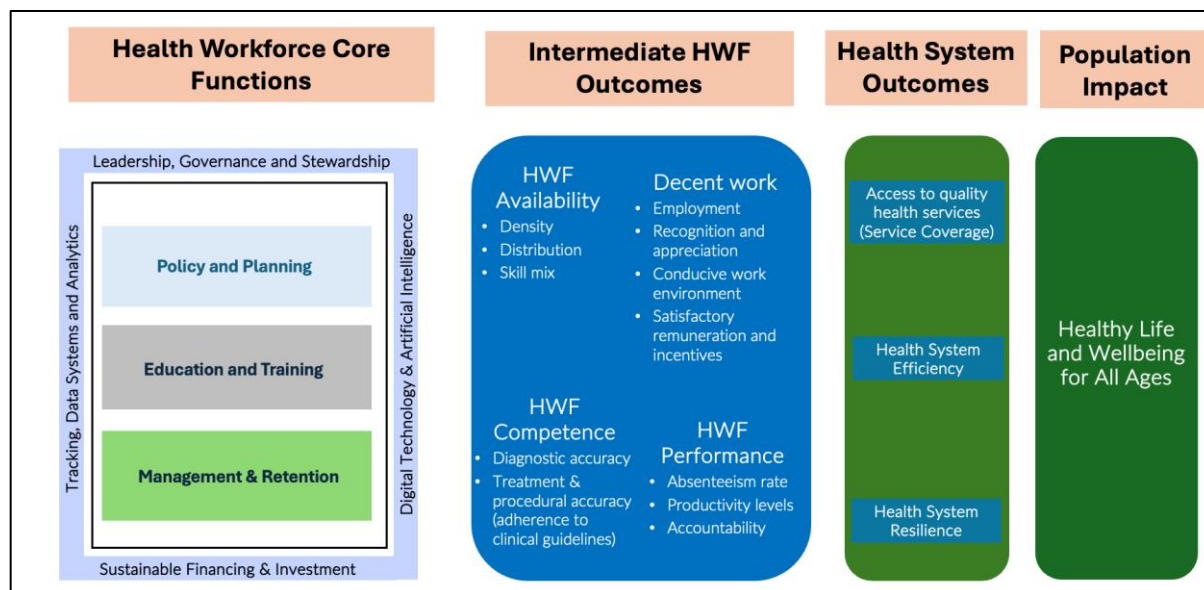


Figure : Theory of Change

The theory of change in Figure X outlines the path to achieve equitable access to skilled and motivated health workers in Africa, leading to "Healthy Life and Wellbeing for All Ages."

GUIDING PRINCIPLES

- Evidence-informed
- Inclusive dialogue
- Needs-based planning and investment
- Competency-based education
- Multisectoral approach
- Efficiency and accountability
- Harmonized Standards
- Right to health services
- Equity

5.4 OBJECTIVES

5.4.1 Objective 1: Operationalize the principles of the Africa Health Workforce Investment Charter to stimulate and sustain evidence-informed health workforce policy reforms and investment informed by current and future needs of the health system and population.

Despite progress in health workforce development over the past decade, Africa faces a projected shortfall of 6.1 million health workers by 2030, which threatens efforts to deliver essential health services. This shortage is exacerbated by suboptimal training capacities and the inequitable distribution of health workers, among other factors. Compounding the crisis, 27% of trained health workers remain unemployed or underemployed, while funding gaps and underutilized private sector potential continue. To holistically address these issues, WHO, Member States and partners launched the Africa Health Workforce Investment Charter to guide governments in mobilizing smarter, sustainable investments. The Charter, adopted by countries in Africa, promotes five evidence-based principles to halve workforce shortages and tackle rural and primary care inequalities by 2030.

Priority Actions for Countries

1. **Conduct Comprehensive Health Labour Market Analysis (HLMA):** Conduct or update comprehensive analyses every 3–5 years, covering workforce supply, demand, need, budget space, and economic and financial feasibility to inform strategic planning, policy reforms, and investments.
2. **Hold National Policy Dialogues to build evidence-informed consensus on health workforce priorities:** Convene major health workforce policy dialogues every 3–5 years informed by recent evidence, supplemented by annual workforce planning and management meetings with stakeholders for course corrections and operational planning.
3. **Develop and Implement National Health Workforce Policy/Strategy and Investment Plan:** Formulate national health workforce policy/strategy and investment plans based on consensus from policy dialogues, guiding short-to-medium term policy reforms and investment priorities as part of (or in line with) national development and health sector plans with specific and feasible targets for health workforce training and employment/recruitment.
4. **Formalize Investment Commitments:** Officially commit to national health workforce investment plans by signing national compacts and integrating commitments into national health budgets, synergistically and efficiently using domestic and official development assistance for health workforce development and recruitment.

5. **Produce Annual Workforce Investment Scorecards:** Annually publish workforce investment scorecards or briefs highlighting progress, challenges, emerging priorities, and actionable opportunities.

5.4.2. Objective 2: Transform and optimise the education and training of health workers to address the population's health needs

Despite a 70% increase in training capacity across Africa, significant gaps persist in the quality, distribution, and local relevance of health worker education. Many curricula remain outdated, not competency-based, and misaligned with countries' current health needs. Educators often lack formal training in pedagogy, and infrastructure continues to be inadequate. Competency gaps among health workers contribute to poor service quality and preventable deaths. To meet population health needs and achieve universal health coverage by 2030, countries must invest in transforming health professions education—by modernizing curricula, strengthening faculty development, and ensuring equitable access to high-quality training that produces skilled, motivated, and practice-ready health workers.

Priority Actions for Countries

1. **Optimise the quality of health professions education:** Adapt and implement competency-based education for pre-service education, specialist training and continuing professional development for all health professions.
2. **Develop and implement national health professions education specific accreditation mechanism:** Formulate national accreditation standards and mechanisms for health professions education and implement quality improvement processes in all health professions education institutions and training platforms.
3. **Establish and sustain mechanisms for the professional development of educators:** Formulate and implement mechanism that support the advancement of educators in both their disciplinary and clinical expertise and in developing robust competencies in the science of health professions education including teaching, learning, assessment, curriculum design and leadership.
4. **Ensure quality in expanding the capacity of education and training of health workers;** Increase students student enrolment aligned with national health workforce needs and supported with concurrent investment in educator capacity and clinical training capacity.
5. **Scale-up and/or optimise health worker education** based on the needs of the local health and population, within strengthened educational institutions.

6. **Establish and sustain mechanisms for capacity enhancement** for educators and faculties to enhance the competency of graduates
7. **Establish and sustain functionality of accreditation mechanisms** and ensure their enforcement across public and private health training institutions.
8. **Increase course enrollment and completion rates** based on nationally defined standards for education of health workers
9. **Strengthen capacities of the public health workforce** in emergency and disaster prevention, preparedness, response and recovery.
10. **Leverage digital technology** in the education, employment, development and retention of health workers

5.4.3. Objective 3: Optimize the employment, retention, management and performance of the health workforce.

Despite progress in training health workers, Africa continues to face a projected shortfall of 6.1 million health workers by 2030. This is due to persistent challenges, including weak employment absorption, poor retention, and suboptimal performance. The shortfall varies across different countries and needs to be carefully estimated, with concerted efforts made to at least halve the contextual deficits and improve working conditions. In many countries, labor market mismatches leave nearly 27% of trained workers unemployed, while poor working conditions and migration drive high attrition rates. Health systems lose up to 20% efficiency due to inequitable distribution of health workers (due to the absence of current health worker staffing norms), absenteeism, and weak performance management. Optimizing workforce policies and plans through strategic recruitment, fair employment practices, equitable distribution based on evidence-based staffing norms, retention incentives, and performance-based management is essential to close this gap, improve productivity, and ensure health workers are effectively deployed and motivated to deliver quality care where it is most needed.

Priority Actions for Countries

1. **Develop Flexible Workload-related National Staffing Norms and Standards:** Collaboratively establish flexible staffing norms based on workload patterns, population health needs, and system capacities, coordinated by national staffing authorities.

2. **Recruitment, Deployment, and Promotion Plans:** Formulate and budget recruitment, deployment, and promotion plans aligned with established staffing norms and standards.
3. **Equitable Workforce Distribution:** Use staffing norms to effectively distribute and deploy newly recruited health workers, prioritizing underserved areas, and regularly monitor workforce distribution equity.
4. **Implement Retention Incentives:** Establish or strengthen targeted incentive packages to attract and retain health workers in underserved areas, and annually track perspectives of health workers through national surveys.
5. **Measure Workforce Productivity and Performance:** Systematically monitor health workforce productivity and performance, using findings to ensure accountability and appropriate recognition or corrective actions.
6. **Enhance Labour Relations and Working Conditions:** Establish and regularly update collective agreements with health worker unions and associations, progressively improving working conditions to enhance workforce retention.
7. **Employee Assistance and Occupational Safety Policies:** Implement comprehensive policies and guidelines for employee assistance programs, occupational health and safety, including health worker insurance schemes addressing occupational risks.
8. **Bilateral Labour Agreements:** Where feasible, establish bilateral labour agreements to sustainably manage health workforce migration.
9. **Progressively integrate community health workers:** Develop community health worker programmes based on health system needs with standardised curriculum for training, clear selection criteria and with scheme of service for clear career progression and integration within the broader workforce.
10. **Develop guidelines for managing workforce during emergencies:** Develop an emergency workforce management guideline governing recruitment, deployment and insurance policy for health workers when there is a formally declared public health emergency.

5.4.4. Objective 4: Build and strengthen leadership and governance capacity for effective health workforce stewardship

Effective stewardship of the health workforce requires strong, capable leadership and robust multisectoral and intra-sectoral governance structures. While many African countries have established dedicated HRH units and policies/strategies/ plans, gaps persist in coordinating efforts, effectively and efficiently implementing initiatives, and securing adequate domestic financing.

Disappointingly, only half of the countries have set aside dedicated funding for their workforce development strategy implementation, and some lack the institutional mechanisms to monitor staffing needs. Strengthening leadership is crucial, as it ensures adequate capacity for planning and management, alignment of workforce policies with national development and health priorities, enhances sectoral coordination, and supports evidence-based planning. Equally vital, robust governance fortifies regulatory systems that safeguard education quality, professional standards adherence, and occupational and workforce safety. Investing in leadership capacity is essential to drive sustainable, accountable, and responsive health workforce development that can truly meet the needs of countries in particular and the continent at large.

Priority Actions for Countries

1. **Strengthen multisectoral collaboration** in health workforce education, employment, management and retention actions through engagement of the executive and parliament to ensure high prioritization of health workforce development
2. **Establish and sustain the functionality of HRH directorates** and units at national and sub-national levels, that minimize fragmentation of the HRH core functions and report to senior management, to develop and monitor HRH development policies and plans, and foster apposite management of the health workforce
3. **Strengthen the leadership and management capacities of HRH directors and unit heads** at national and sub-national levels to coordinate and facilitate intersectoral health workforce agenda.
4. **Undertake a benchmarking** of the level of health workforce development, including education, utilization and retention, data, leadership and governance, using appropriate tools.

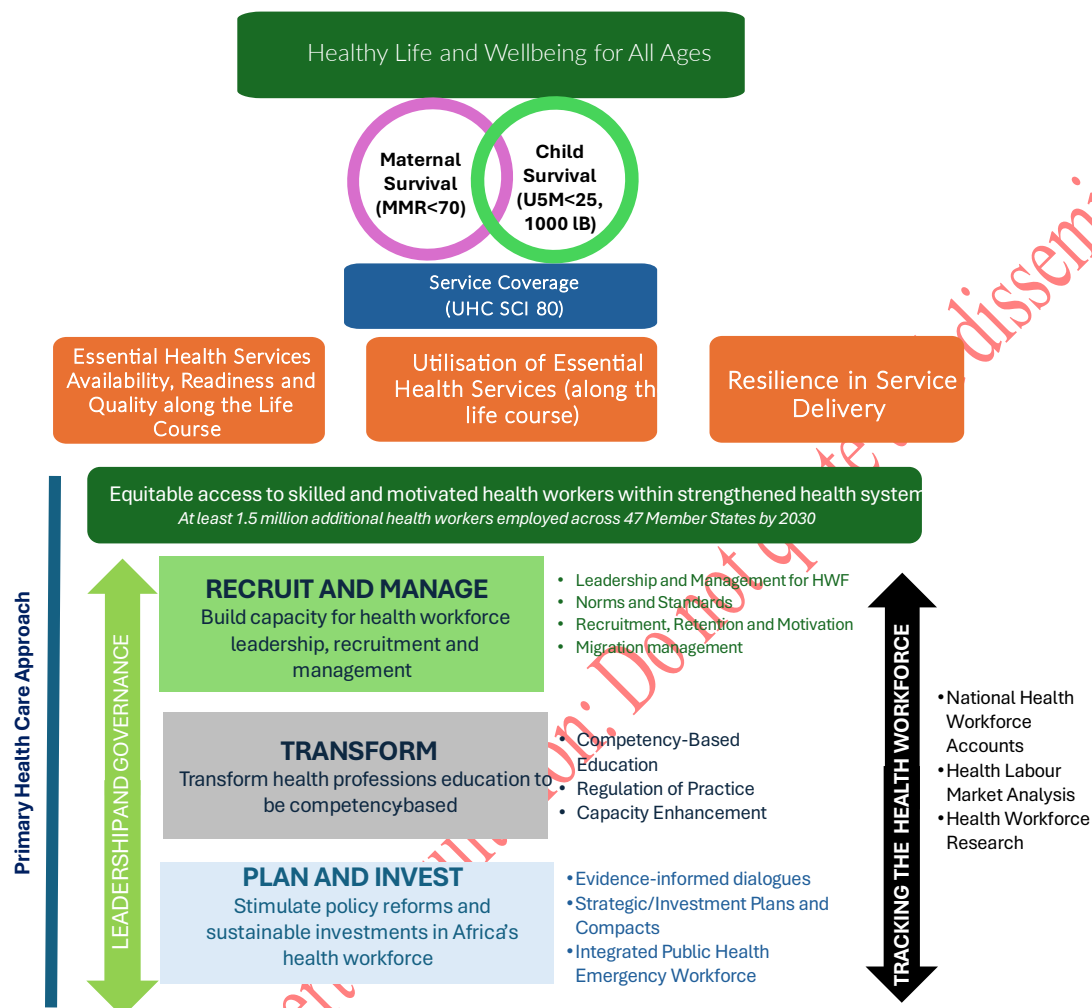
5.4.5. Objective 5: Track the labour market dynamics and improve systems for availability of health workforce data and evidence for decision-making and accountability.

Robust and comprehensive health workforce data and evidence are crucial for effective and evidence-based planning, policymaking, and accountability. Many countries face challenges with fragmented information systems and limited labor market intelligence, deterring their ability to align workforce supply, demand, and population health needs. Strengthening data systems, such as national registries and the National Health Workforce Accounts, allows governments to monitor workforce trends, identify gaps, and forecast future requirements and skill mix. Tracking labor market dynamics, including macroeconomic context, education, employment, migration, and effective supply, supports evidence-based decisions at national and sub-national levels on recruitment, training, and retention. Investing in robust data systems based on nationally agreed-upon datasets tracked using various mechanisms promotes transparency, enhances multisectoral coordination, and empowers stakeholders to make informed, human-centered decisions that strengthen health systems and workforce resilience.

Priority Actions for Countries

1. **Establish or Strengthen HR Information Systems:** Establish and sustain functionality of national HRH information systems, including registries for employed and practicing health workers based on national minimum datasets and standardized data flow process to systematically manage data and track workforce movements from education entry through employment.
2. **Implement National Health Workforce Accounts (NHWA):** Establish a multisectoral coordinating group to implement NHWA to track the Health Labour Market Dynamics, including having data sharing mechanisms to enable progressive improvement in reporting on core indicators
3. **Analyze and disseminate annual information on health workforce performance,** including vacancies and staffing levels, to inform education and recruitment of health workers.

CHAPTER 6: HOW WHO WILL SUPPORT COUNTRIES TO DELIVER: THREE STRATEGIC INITIATIVES



Strategic Initiative 1: Stimulate policy reforms and sustainable investments in Africa's health workforce - INVEST

Africa's health systems face persistent and pressing challenges, including critical shortages of health workers, inequitable distribution - especially in rural and primary care settings - and chronic underinvestment in workforce development. Addressing these challenges demands not just more funding, but smarter, more strategic investment. This recognition informed the creation, launch, and implementation of the Africa Health Workforce Investment Charter, adopted through the Windhoek Statement on Investing in Africa's Health Workforce at the inaugural Africa Health Workforce Investment Forum in May 2024.

The Charter provides a transformative framework to guide governments in reducing the 6.1 million health workforce shortages by half and tackling inequalities in access to care. It builds on a 2017 Regional Committee resolution setting a 2030 target for all Member States to cut inequalities in access to health workers by at least 50%. Central to this vision is the principle of mobilizing and sustaining political and financial commitment, while fostering cross-sectoral collaboration to improve the development, performance, and retention of the health workforce.

Urgent action is needed to drive policy reforms and sustainable investments, moving away from fragmented, short-term interventions towards long-term, systemic solutions. Historically, investments in Africa's health workforce have been reactive, focusing on in-service training or temporary incentives - an approach that has proven insufficient to meet the growing demand for health services, particularly in underserved areas.

The Charter outlines five evidence-based investment principles to guide countries: government leadership, data-driven prioritization, alignment and synergy, stimulation of investments, and sustainability. These principles shape a structured investment process starting with identifying and prioritizing opportunities based on robust data. Governments and partners then engage in investment dialogues to align resources and negotiate commitments, which are formalized in investment plans and compacts to ensure accountability. This approach aims to not only mobilize investments, but also ensure they are effectively coordinated and targeted. It encourages governments to reduce inefficiencies in current spending and increase domestic and external resources for health workforce development. Crucially, the Charter emphasizes linking investments to measurable health outcomes and inclusive economic growth, so the returns are both social and economic.

Priority intervention towards *accelerating efforts to halve the 6.1 million HWF shortage by optimizing training outputs, especially at the primary healthcare level, where the return on investment is greater.*

1. Conduct a comprehensive Health Labour Market Analysis (HLMA) in 25 countries
2. Tracking Health Labour Market Dynamics using National Health Workforce Accounts (NHWA)
3. Undertake evidence-informed multi-sectoral dialogues to inform Policy reforms in HEF education, employment and Investment planning.
4. Develop Health Workforce Policies and Strategic Plans, including Health Workforce Investment Plans and Compacts to formalize commitments.
5. Develop action plans to integrate the Public Health and Emergency Workforce within national workforce development programmes.

INVEST



- Conduct comprehensive Health Labour Market Analysis (HLMA) in 10 countries
- Routinely track workforce dynamics using National Health Workforce Accounts (NHWA)
- Undertake evidence-informed multi-sectoral dialogues to inform Policy reforms in HEF education, employment and Investment planning.
- Develop Health Workforce Policies and Strategic Plans, including Health Workforce Investment Plans and Compacts to formalise commitments.
- Develop action plans to integrate the Public Health and Emergency Workforce within national workforce development programmes.

6

Strategic Initiative 2: Transform health professions education in Africa - TRANSFORM

Achieving a quality health workforce requires the transformation of education and training strategies. The strategy for transformation is an opportunity to reimagine how health workers are educated including the educators, the location of their training, the approaches towards education and the relevance of the education to health outcomes.

The WHO is committed to supporting Member States in strengthening the quality, relevance, and capacity of health professions education. Central to this is the adoption of competency-based education (CBE) as a foundation for the education of health workers who are responsive to population and health system needs. Through the Global Competency and Outcomes Framework for Universal Health Coverage, and the development of Regional Prototype Competency-Based Curricula for eight priority health professions, WHO AFRO will provide a model for countries to adapt and adopt. In supporting Member States to institutionalise competency-based education, WHO will facilitate national dialogues on health professions education, guide the development of national implementation roadmaps, and promote the use of context-appropriate strategies for effective adoption and implementation of CBE across pre-service, in-service, and specialist training programmes.

Ensuring education quality also requires robust quality assurance and accreditation mechanisms. WHO will work with countries to advance fit-for-purpose accreditation systems that address all aspects of education quality — including institutional governance, teaching and learning processes, educator capacity, assessment practices, and student outcomes. Defining clear standards is essential — but standards alone do not guarantee quality, as there is a need for health professions education institutions to be supported to meet and sustain these standards, through well-aligned investments and quality improvement processes.

Educator professional development is equally critical to the transformation of the health workforce. Many educators lack formal preparation in health professions education, which undermines the effectiveness of CBE. WHO will support Member States in developing faculty development standards and programmes to ensure educators are equipped to lead this

transformation. This includes continuing professional development in educational methods, assessment, leadership, and scholarship.

Expanding the capacity of health professions education institutions is essential to meet future health workforce needs. The expansion must be done responsibly — by aligning student enrolment with national health workforce plans, strengthening completion rates, and expanding the diversity of clinical learning platforms across the health system. WHO will work with countries to reflect enrolment standards, ensure alignment with labour market needs and population health priorities, and support quality assurance mechanisms to safeguard education quality as capacity is expanded.

Priority outputs towards *transforming the health workforce through education*

1. Support countries in adopting and adapting the regional prototype competency-based curricula
2. Develop harmonised regional standards to support the quality assurance mechanisms for health professions education
3. Establish mechanisms to support the professional development of educators in health professions education institutions
4. Support the development of master health profession education plans to expand and optimise enrolment that match population health needs and health system capacity.



TRANSFORM

- Develop Regional Prototype Competency - Based Education Curricula for 15 priority occupations.
- Conduct national benchmarking and adaptation of Prototype Competency Based Education Curricula in 10 countries.
- Develop a standardised tool for conducting Training Needs Assessment
- Develop harmonized regional guidelines for accreditation and standards for health professions education and training
- Establish Educator Capacity Improvement Support Programme

7

Strategic Initiative 3: Build leadership and management capacities for effective health workforce stewardship - BUILD

Strong leadership, effective recruitment, deployment, and retention, and sound management are the backbones of a resilient and responsive health workforce. Addressing Africa's evolving health workforce challenges requires improved capacity of health workforce leaders and managers to

drive reforms, optimize human resources for health (HRH), and accelerate progress toward Universal Health Coverage (UHC). HRH leaders, occupying strategic positions, must be capable of aligning workforce strategies with national health priorities, mobilizing resources, and fostering collaboration across sectors. Building such leadership capacity requires structured, innovative training programs that blend technical knowledge with strategic thinking, policy development, and stakeholder engagement. It also requires designated and strengthened HRH directorates or units with the needed human and material resources.

Recruitment and deployment are equally critical. Many African countries face persistent challenges in attracting and retaining qualified health workers, particularly in rural and underserved areas, often due to the lack of evidence on needs and evidence-based national staffing norms. These challenges are often exacerbated by weak human resource planning, a lack of data-driven decision-making, inefficient recruitment systems, and high migration trends. Strengthening recruitment capacity involves not only improving the processes for identifying and hiring qualified personnel but also ensuring that recruitment is equitable, transparent, and aligned with service delivery needs informed by local evidence. This includes the use of evidence from health labor market analyses, workload indicators of staffing needs, and competency-based recruitment frameworks to ensure the right people with the right skills and right attitude are placed in the right roles.

Effective management of the health workforce is essential to ensure that health workers are supported, motivated, and productive. This can be achieved by having contextual performance management, continuing professional development, and a supportive supervision system, as well as enabling work environments.

Investments in this area must be sustained, evidence-based, and integrated into broader health sector reforms as they unlock the full potential of the health workforce, create a supportive work environment, and foster decent work. Ultimately, they enable countries to address workforce challenges proactively, improve health service delivery, and build resilient health systems.

Priority outputs towards *building leadership and management capacities for effective health workforce stewardship*.

1. HRH Leadership and Management capacity building and strengthening, including for directorates, supportive supervision
2. Institutional Health Workforce Capacity Strengthening for Programs and Practice Regulation
3. Staffing Norms and Standards for recruitment planning and equitable distribution
4. Precision health workforce development and distribution informed by productivity and national health workforce surveys
5. Guidelines for Effective HRH Management, including recruitment, retention, performance and motivation, especially for underserved and rural areas.
6. Health Worker International Migration Management



Build leadership and management capacities for effective health workforce stewardship

To enhance management at all levels and ensure decent jobs and proper retention for health and care workers, especially youths and women.

BUILD

- HRH Leadership and Management capacity building and Strengthening, including for directorates, supportive supervision
- Institutional Health Workforce Capacity Strengthening for Programs and Practice Regulation
- Staffing Norms and Standards for recruitment planning and equitable distribution
- Precision health workforce development and distribution informed by productivity and national health workforce surveys
- Guidelines for Effective HRH Management, including recruitment, retention, performance and motivation, especially for underserved and rural areas.
- Health Worker International Migration Management

8

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CHAPTER 7: ROLES AND RESPONSIBILITIES IN ACTUALIZING THE STRATEGIC INITIATIVES

Member States

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WHO

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Development partners

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CHAPTER 8: MEASUREMENT FRAMEWORK

Indicator	Indicator Definition	Disaggregation	Level
Leadership, Governance and Stewardship			
Existence of a health workforce directorate/ unit in the Ministry of Health responsible for developing and monitoring policies and plans on health workforce	Existence of national health workforce directorates/ units with direct reporting to health sector leadership (Yes/Partial/No)		National Sub-national
Existence of institutional mechanisms or bodies to coordinate an intersectoral health workforce agenda and policy/investment dialogues (Yes/Partial/No)	Assesses whether a formal and functional intra/intersectoral committee/ technical working group for HRH coordination is in place	N/A	National Sub-national
Existence of institutional models for assessing and monitoring staffing needs for health service delivery (Yes/Partial/No)	Presence or establishment status of institutional models for assessing and monitoring staffing needs		National
Protocol for surge HW recruitment	Availability of national or subnational protocols for rapid recruitment of surge health workers during emergencies		National/Sub national

Indicator	Indicator Definition	Disaggregation	Level
Roster of rapid response team available	Functional, up to date (within the last 12 months) roster of a readily available multidisciplinary rapid response team for emergency response and surge capacity in place	Subnational	National; subnational
Current state of delivery of the essential public health functions (EPHFs) has been ascertained	A comprehensive review of Essential Public Health Functions (EPHFs) has been done within five years, assessing prioritization, integration into national plans, coordination mechanisms, and monitoring and evaluation systems, ensuring alignment with population health needs, cross-sector strategies, and follow-up actions based on monitoring outcomes.		National
Sustainable Financing and Investment			
Government health workforce compensation budget as a percentage of government health budget	Measures health worker compensation share (%) of total government health budget		National Sub-national
Government expenditure on compensation of health workers as a percentage of the government health expenditure	Measures the share (%) of government health expenditure spent on health worker compensation		National
Total expenditure on compensation of health workers	Measures the total annual expenditure (absolute) on health worker compensation	Public vs private	National
Tracking labour market dynamics , data systems and analytics			
Health Workforce data reporting completeness	Measures the percentage of national submitting entities (health facilities, regions, administrative units etc.) regularly reporting key workforce data	National submitting entities	National
Health workforce priority setting is informed by health labour market data and evidence	Assesses the conduct and use of national health labour market analysis findings to inform National health/health workforce policies/ strategies/ AOPs	Health occupations	National
Existence of an M&E framework for national health workforce tracking and reporting	Assesses the existence of framework for annual HW tracking and reporting through annual health workforce performance reports (Yes/No)	Subnational	National
Functional national human resource information system and national health workforce accounts	Assesses the presence of functional and operational HRHIS and reporting of NHPWA indicators annually	Subnational Health professions	National

Indicator	Indicator Definition	Disaggregation	Level
National capacity to monitor key metrics for health workforce planning and global monitoring frameworks	Ability of HRHIS (or other mechanism) to generate information to report on health workforce metrics for International Health Regulations, WHO Global Code of Practice on International Recruitment of Health Personnel, outputs from education and training institution, track health labour market	Type of report	National
Digital Technology and Artificial Intelligence			
Percentage of health providers providing telemedicine services	Measures the proportion of health providers who use telemedicine in service delivery	Health professions Public vs Private Facility type	National Subnational
Percentage of facilities using electronic health workforce management systems	Measures the share of facilities with digital systems for managing HRH data and information	Facility type Sub-national	National
National capacity to generate geocoded information on the location of health facilities and staffing levels	Capacity of national human resources for health information systems to generate information on location of functional health facilities and staffing levels for equity analysis	Facility type Sub-national	National
Policy and Planning			
Existence of national HRH policy/strategy oriented to PHC, UHC and national health development plan	Assesses the availability of national HRH policy aligned with PHC and UHC goals, and aligned to the national health development plan (Yes/No)	Sub-national	National
Existence of national HRH annual operational plans informed by national health/HRH policy/strategy	Assesses the availability of annual operational plans aligned with national HRH strategy (Yes/No)	Sub-national	National
Implementation rate of the national health workforce strategic plan	Measures the proportion of activities completed vs planned in national health workforce strategic plan period	Activity Subnational	National
Implementation rate of the national health workforce investment plan and compact	Measures the proportion of investment plan targets achieved within timeframe	Intervention area	National
Education and Training			
Existence of national education plans for the health workforce, aligned with the national health plan and the national health workforce strategy/plan	Assesses whether the national education plans align with national health development and health workforce strategy (Yes/No)	health profession	National
Master list of accredited health workforce education and training institutions	Assesses the availability of an up-to-date master list of accredited training institutions.	health profession institution type Subnational	National

Indicator	Indicator Definition	Disaggregation	Level
Percentage of education programs accredited	Measures the percentage of health workforce education programs with official accreditation to train health professionals	health profession institution type Subnational	National
Annual health worker graduates per 10,000 population (all cadres)	Tracks the number of health worker graduates per year per 10,000 population.	health profession	National
Existence of national and/or subnational mechanisms for accreditation of health workforce education and training institutions and their programmes	Assesses whether official accreditation mechanisms exist for training institutions/programs.	health profession institution type	National
Existence of national systems for continuing professional development	Assesses whether there is a national framework to manage mandatory CPD for licensed health professionals. (Yes/No)	Health profession	National
Existence of national prototype competency-based curricula for health professions education	Assesses whether a national prototype competency-based curriculum framework exists to guide pre-service health professions education (Yes/No).	health profession	National
Ratio of graduates to stock	Measures the ratio of graduates of health workforce education and training programmes to the stock of active health workers	Health workforce education and training programme health workforce education and training programme and sex health workforce education and training programme and health education and training institution ownership (public/private)	National
Management and Retention			
Existence of national licensing and regulation system	Assesses whether a national regulatory authority/licensing board for health professions exists to ensure quality and safety.	health profession	National
Continuing professional development (CPD) system in place	Assesses whether a national framework exists to require and manage CPD for licensed health professionals (Yes/No).	health profession	National
Annual inflows of health workers	Tracks the number of new health workers entering the workforce each year, relative to total stock.	cadre sex place of training (domestic/foreign).	National
Annual outflows of health workers	Tracks the number of health workers exiting the workforce each year, due to retirement, migration, death, or other reasons.	By cadre, exit reason	National

Indicator	Indicator Definition	Disaggregation	Level
Existence of national health workforce staffing norms based on workload analysis	Whether national staffing norms or standards exist based on workload, professional standards and service utilization (Yes/Partial/No).	cadre service level	National
Existence of scopes of practice with roles and functions	Whether official scopes of practice are defined and endorsed for each health profession (Yes/Partial/No).	By cadre	National
Existence of mechanisms/ schemes for in-kind remuneration to promote rural retention (Yes/Partial/No)	Whether there are non-financial incentive schemes (e.g., housing, transport, hardship allowances in kind) to attract/retain health workers in rural or underserved areas.	Cadre Subnational	National
Health Workforce Availability			
Density of SDG 3c Occupations	Measures the number of practicing doctors, nurses, midwives, dentists, and pharmacists per 10,000 population — a core measure of workforce availability for the SDG 3c occupations.	health profession	National
Health worker distribution by age group	Measures the percentage of health workers in defined age groups (e.g., < 25, 25–34, 35–44, 45–54, 55–64, ≥ 65 years) — indicates workforce age structure and renewal capacity.	Health profession Sex	National Subnational
Health worker distribution by sex	Measures the percentage of active health workers by sex.	health profession	National Subnational
Health worker distribution by facility ownership	Measures the percentage of all health workforce employed by insitutional sector (public or private not-for-profit or private for-profit)	health profession	National Subnational
Health worker distribution by facility type	Measures the percentage of health workers by type of facility where they work (e.g., hospital, health centre, lab).	health profession	National Subnational
Vacancy Rate	Measures the percentage of unfilled posts to total number of funded posts for ≥12 months	Health profession Sex Facility type	National, Subnational, Facility
Decent Work			
Annual outflows of health workers	Tracks the number of health workers exiting the workforce each year, due to retirement, migration, death, or other reasons.	Health profession Exit reason	National

Indicator	Indicator Definition	Disaggregation	Level
Existence of national/subnational policies/laws regulating working hours and conditions	Assesses whether formal legal frameworks, policies or labour laws exist defining and protecting standard working hours, leave entitlements, rest periods, schemes of service and conditions for health workers. (Yes/No)	Sector	National, Subnational Facility
Percentage of facilities with access to or being covered by dedicated services for occupational safety and health	Measures the percentage of facilities being covered by services for occupational safety and health hazards, identified within their context (for example, occupational health, health and safety services, including services for those health and social care workers affected by public health emergencies or events, etc.). Arrangements for occupational safety and health services established (for example, services in facility; services provided by local health authorities) can be considered as facilities being covered.	Facilities Subnational	National, Subnational Facility
Health Workforce Competence			
Availability of human resources to implement International Health Regulation core capacity requirements (None/Limited/ Developed/ Demonstrated/ Sustainable)	Self-assessed level of national capacity to mobilize and deploy sufficient trained health workforce to meet IHR core capacities for emergency preparedness and response.	Sector (human health, veterinary, border health)	National
Rate of a specific healthcare associated infection (HAI) (which is relevant to the country context) in patients	Incidence or prevalence rate of a defined HAI per 1,000 patient days or discharges (e.g. surgical site infection, catheter-associated infection).	Facility type Subnational Patient demographics (age, sex)	Facility Subnational
General readmission rate at the facility (hospital readmission within specified days of discharge)	Measures the percentage of patients readmitted to the same or another hospital within a defined timeframe (e.g., 30 days) after discharge	Facility type Cause of readmission Patient age group	Facility Subnational
Rate of hospital acquired infections (HAIs)	Measures the number of new hospital-acquired infections per 1,000 patient days or admission	Facility type Infection type	Facility Subnational
Health Workforce Performance			
Health worker absenteeism rate	Measures the percentage of workdays health workers are absent from duty	Health profession Facility level	Facility Subnational
Productivity (e.g. patient consultations per health worker per year)	Measures the average number of patient visits or services delivered per health worker per year	By cadre; facility	

Indicator	Indicator Definition	Disaggregation	Level
Vacancy Rate	Measures the percentage of unfilled posts to total number of funded posts for ≥ 12 months	Health profession Sex Facility type	National, Subnational, Facility
Average length of stay (ALOS) at hospital or health facility (average number of days a patient has stayed at the facility from admission to discharge)	Measures the average number of days a patient stays in the facility from admission to discharge.	Facility type Age group	National, Subnational, Facility
Average number of consultations per capita per year	Measures the average number of outpatient consultations provided per person per year.	Sex Facility type Subnational	National
Access to quality health services			
Percentage of population covered and are benefiting from nationally defined essential health service packages.	Measures the share of the population covered by nationally defined essential health service packages	Age cohorts Subnational	National Subnational
Skilled birth attendance rate	Measures the percentage of births attended by skilled personnel (doctors, nurses, midwives)	Facility type Subnational	National Subnational
Percentage of children fully immunized	Measures the share of children that are fully immunized	Age cohorts Subnational	National Subnational
Percentage of facilities offering services according to nationally defined essential health service package for their level of care	Measures the percentage of health service delivery facilities offering essential health services package based on the nationally defined service package	Facility type Ownership	National Subnational
Percentage of population that can access primary healthcare (PHC) within one hour's walk from dwellings (or live within 5km of a PHC facility)	Measures the share of the population within 5km of a PHC facility	Facility type Ownership	National Subnational
Health System Efficiency			
Health worker absenteeism rate	Measures the percentage of workdays health workers are absent from duty	Health profession Facility level	Facility Subnational
Provider caseload	Measures the average number of outpatient services provided by a given health worker in a specified period (e.g., working day, year)	Facility type Subnational Urban/rural Ownership Health profession	National Subnational Facility level
Bed occupancy	Measures the percentage of available beds that have been occupied over a given period	Facility type Subnational Urban/rural Ownership Health profession	National Subnational Facility level
Health System Resilience			

Indicator	Indicator Definition	Disaggregation	Level
Percentage of facilities that closed/discontinued services during an emergency	Measures the share of health facilities that were closed or discontinued provision of essential health services during a public health emergency	Facility type Subnational Urban/rural Ownership Health profession	National Subnational Facility level
Roster of rapid response teams available	Assesses the availability of a nationally approved roster for rapid response teams	Subnational Health profession	National Subnational
Health system resilience measured and monitored in routine health information system	Assesses whether health system resilience is being measured and monitored as part of routine health information system.	Subnational Urban/rural	National Subnational
Healthy Life and Wellbeing for all Ages			
Health-Adjusted Life Expectancy (HALE) at birth SDG3	Measures the Average number of years that a person can expect to live in "full health" by taking into account years lived in less than full health due to disease and/or injury.	Age Sex	National Subnational
Maternal mortality ratio SDG 3.1.1 <i>Reduce MMR to less than 70 deaths per 100,000 live births</i>	Measures the he number of maternal deaths during a given time period per 100 000 live births during the same time period.	Age Subnational	National Subnational
Under-5 mortality rate SDG 3.2.1 <i>Reduce under-five mortality to less than 25 deaths per 1,000 live births</i>	Measures the probability of a child born in a specific year or period dying before reaching the age of five, if subject to age-specific mortality rates of that period. Under-five mortality rate as defined here is strictly speaking not a rate (i.e. the number of deaths divided by the number of populations at risk during a certain period of time) but a probability of death derived from a life table and expressed as rate per 1000 live births.	Age Subnational	National Subnational
UHC service coverage index	Composite index of essential service coverage (SDG 3.8.1)	Wealth quintile Subnational	National Subnational

REFERENCES